

Policies & Procedures (2026) For Crescent Community Care Services Limited

Care Staff

Absenteeism Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To manage absenteeism in such a way as to minimise instances of absenteeism and the effects of unavoidable absenteeism.

Scope

- All employees.

Policy

- Avoidable absenteeism, such as short notice unexplained/un-evidenced sickness, will not be tolerated and may lead to disciplinary action.
- Targets for acceptable absenteeism, such as sickness, will be set (e.g. national average sickness rates) and employees assessed against those targets.
- All absenteeism will be recorded on the personnel file, and the information will be:
 - Reviewed, assessed and discussed during formal supervisions, where acceptable personal targets will be set;
 - Passed to future employers.
- Staff will be interviewed on returning from every instance of absenteeism.

Procedure

- Staff will receive instructions, via the Employee Handbook, about the conduct expected with respect to absenteeism. In general, the organisation should be informed of the intended absence as soon as possible on the first day of absence. If appropriate, the duty manager of the establishment where the employee was expected to work must be informed directly. Failure to do so may lead to disciplinary action.
- An absence form is to be completed for every instance of absenteeism, immediately on return to work. In the case of absence due to sickness, the appropriate form (attached to the Sickness Absence Policy and Procedure) should be used; in the event of absence for any other reason, the form attached below must be used.
- The absence form will be presented to the manager, who will interview the employee regarding the absence and agree with them a strategy for avoiding future absences.
- Unauthorised absences will not be paid for, and may not be converted into holiday pay unless agreed by the manager during contact on the first day of absence.

Reviewed by Marcus Kerridge McColl
December 2025

Accident and Incident Reporting Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards.
- To ensure that accidents and incidents are recorded and analysed in order to satisfy health and safety requirements and provide information for the minimisation of risks.
- All persons affected by accidents or incidents.

Policy

- Accidents which cause injury to any person on the organisation's premises, or employees on duty but not on the premises, or Service Users at any time, will be recorded to promote analysis and management of risks.
- The appropriate recording book is "Accident Book B1510", obtainable from good bookshops and Amazon.

Procedure

- In the event of an accident, or an incident (e.g. a "near miss" which did not result in an injury but which may have done so in different circumstances) an accident/incident form should be completed and immediately submitted to the Registered Manager, who, after review of the facts, will take those actions necessary to minimise danger of the same accident/incident in future. These actions should be noted on the form, using the reverse if necessary. The progress of the treatment of any injury must also be recorded, together with any final outcomes evident at the time of completion and transmission of the form. The Registered Manager must sign the form on completion of the investigation in order to denote that they have discharged their responsibility.
- If the accident or incident involves a Service User, the Duty of Candour Policy and Procedure should also be referred to and, if applicable, followed. This policy can be located in the Administration section of your QCS system.
- After the accident/incident investigation and all matters concerned with it are complete, a copy of the signed accident/incident form should be placed in the personal file of any person(s) affected by the accident, and the original placed in the accident book in the department. In the case of employees the record must be kept on their personnel file, and in the case of Service Users, kept on the Care Plan.
- The purpose of the records held in the department is to provide a chronological, ordered record of accidents.
- The intention of the records placed in the personal file(s) is to provide a record of accidents organised by person, and another record organised by accident type.
- Registered Manager is responsible for the recording of accidents in accordance with RIDDOR (Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013), by:
 - Ensuring that this policy and procedure is followed and all accidents are recorded, using the form attached;
 - Ensuring that all accidents which result in absence from work for more than seven days (not including the day of the accident) are reported within 10 days to the Health and Safety Executive;

RIDDOR 2013 reporting requirements

Reportable incidents under RIDDOR 2013 are: §

- Death and injuries where:
 - The accident is [work-related](#);
 - It results in an injury of a type which is reportable. §
- The types of reportable injuries are:
 - Death;
 - Specified injuries, which are defined by the HSE as:
 - Fractures, other than to fingers, thumbs and toes.
 - Amputations.

- Any injury likely to lead to permanent loss of sight or reduction in sight.
 - Any crush injury to the head or torso causing damage to the brain or internal organs. CE
 - Serious burns (including scalding) which:
 - Cover more than 10% of the body;
 - Cause significant damage to the eyes, respiratory system or other vital organs. CE
 - Any scalping requiring hospital treatment.
 - Any loss of consciousness caused by head injury or asphyxia.
 - Any other injury arising from working in an enclosed space which:
 - Leads to hypothermia or heat-induced illness;
 - Requires resuscitation or admittance to hospital for more than 24 hours.
- The following occupational diseases are reportable:
 - Carpal tunnel syndrome;
 - Severe cramp of the hand or forearm;
 - Occupational dermatitis;
 - Hand-arm vibration syndrome;
 - Occupational asthma;
 - Tendonitis or tenosynovitis of the hand or forearm;
 - Any occupational cancer;
 - Any disease attributed to an occupational exposure to a biological agent.
 - Dangerous occurrences require reporting, for example:
 - The collapse, overturning or failure of load-bearing parts of lifts and lifting equipment;
 - Plant or equipment coming into contact with overhead power lines;
 - Explosions or fires causing work to be stopped for more than 24 hours.
 - For full details of reporting requirements, go to the HSE website at: <http://www.hse.gov.uk/riddor/reportable-incidents.htm>
 - Contacts for the HSE, including for online reporting can be found at: <http://www.hse.gov.uk/contact/contact.htm>
 - **Terminology:**
 - An accident is an incident which results in an injury to a person;
 - An incident is similar, but did not on that occasion result in injury, but is required to be recorded and investigated in order to prevent a recurrence and possible injury in the future;
 - An incident could also include an event which led only to property damage;
 - Persons filling in the reporting form should not be concerned about differentiating between an incident and an accident if the allocation is unclear – the Registered Manager will complete the allocation on review of the form.

Reviewed by Marcus Kerridge McColl
November 2025

Alcohol and Drugs Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To maintain safe and effective working conditions.

Scope

- All workers.

Policy

- Crescent Community Care Services Ltd will work with employees and other workers to ensure that their use of either alcohol or drugs does not impair the safe and efficient running of the organisation or the health of its employees.

Procedure

THE WARNING SIGNS!

- Supervisors and managers should be aware that the misuse of drugs or alcohol by employees could come to light in various ways. The following characteristics, especially when arising in combinations, may indicate the presence of an alcohol or drug-related problem.

ABSENCES

- Instances of unauthorised absences.
Frequent Friday and/or Monday absences.
Leaving work early.
Lateness (especially on returning from lunch).
Excessive level of sickness absence.
Strange and increasingly suspicious reasons for absence.
Unusually high level of sickness for colds, flu, and stomach upsets. § Unscheduled short-term absences, with or without explanation.

HIGH ACCIDENT LEVEL

- At work.
- Elsewhere, i.e. driving, at home.

PERFORMANCE

- Difficulty in concentration.
- Work requires increased effort.
- Depression.
- Irritability.
- General confusion.
- Individual tasks take more time.
- Problems with remembering instructions or own mistakes.

MOOD SWINGS

- Irritability

- Depression
- General confusion.

MISCONDUCT

- An employee's alcohol or drug problem may come to light as a mitigating factor in a disciplinary interview. It should be treated as a mitigating factor for certain "less serious" disciplinary offences, such as poor time-keeping or lateness, provided the person is prepared to undergo treatment.

SELF-REFERRAL

- In some instances, employees may seek help and advice themselves.

INTERVENTION

- Supervisors/managers who feel that an employee's unsatisfactory performance may be drug or alcohol-related should inform their immediate supervisor to discuss the matter. If thought necessary, the department manager should arrange for a meeting with the employee. At the meeting the individual should have their unsatisfactory performance/conduct and "indicators" discussed.
- The department manager should restate the organisation's required standards, making sure the employee understands what is expected of them.
- The department manager should try to establish the cause of the problem (although it must be pointed out that individuals with a drugs or alcohol problem will often go to great lengths to conceal the situation).
- The employee should be reminded or informed of the assistance the organisation is prepared to give employees who are trying to overcome a drink or drugs problem and should be informed of outside agencies at which help can be obtained. The employee should be informed that the organisation requires that their performance be improved to an acceptable standard and that failure to achieve this will result in dismissal.
- The department manager should agree with the employee as to what follow-up action should be taken.
- Where it is established that alcohol or drugs are or could be the problem, an appointment should be arranged with the organisation's Medical Advisor or an independent medical practitioner.
- If the employee denies that either alcohol or drugs are the cause of the problem (whether believed or not), he or she should be treated the same as they would for any other disciplinary/capability problem, whichever is judged as appropriate by the employer. However, if there are strong signs that the employee's unsatisfactory performance is drug or alcohol-related, and they will not admit or acknowledge this, further encouragement should be given at all stages of the disciplinary/capability procedure for the individual to face up to the true, underlying problem(s).

TREATMENT

- Where employees acknowledge that they have a problem and are given help and treatment, this will be on the understanding that:
 - While they are undergoing treatment, they will be on sick leave and will be entitled to the usual Organisation sick pay benefits.
 - Every effort should be made to ensure that on completion of the recovery programme employees are able to return to the same or equivalent work.
- However, where such a return would jeopardise either a satisfactory level of job-performance or the employee's recovery, the department manager will review the full circumstances surrounding the case and agree a course of action to be taken. This may include the offer of suitable alternative employment, the consideration of retirement on the grounds of ill health, or dismissal. (Before a decision on dismissal is made, it should be discussed with the employee and an up-to-date medical opinion should be obtained – see procedure for dealing with ill-health dismissals).

RELAPSE

- Where an employee, having received treatment, suffers a relapse, the Organisation will consider the case on its individual merits. Medical advice will be sought in an attempt to ascertain how much more treatment/rehabilitation time is likely to be required for a full recovery. At the organisation's discretion, more treatment or rehabilitation time may be given in order to help the employee to recover fully.

RECOVERY UNLIKELY

- If, after employees have received treatment, recovery seems unlikely, the organisation may be unable to wait for the employee any longer. In such cases a clear warning will be given to the employee beforehand, and a full medical investigation will have been undertaken.
- In certain cases, strictly at the discretion of management, early retirement may be considered.

SERIOUS MISCONDUCT CAUSED BY ALCOHOL OR DRUGS

Intoxicated employees:

- If an employee is known to be, or is strongly suspected of being, intoxicated by alcohol or drugs during working hours, the department manager will be consulted. Arrangements will be made for the employee to be escorted from the company premises immediately. Disciplinary action will take place when the employee has had time to sober up.

Consumption of alcohol on the premises

- Employees are, under normal circumstances, expressly forbidden to bring or consume alcohol on the premises. Any breach of this rule will result in disciplinary action being taken which is likely to result in summary dismissal. There are, however, certain occasions when the management will make an exception, for example at the annual Christmas party. Such occasions will be clearly notified to employees in advance via the organisation notice boards. At such times, employees will be expected to behave in a responsible manner.
- Drivers are advised, where necessary, to find alternative means of transport home.

DRUG ABUSE ON THE PREMISES

- Employees who take drugs which have not been prescribed on medical grounds will, in the absence of mitigating circumstances, be deemed to be committing an act of gross misconduct and will thus render themselves likely to be summarily dismissed as will any employee believed to be buying or selling drugs, or in possession of unlawful (i.e. non-prescribed) drugs.

TRAINING AND DEVELOPMENT

- Supervisors and managers will be given training in order to develop "early recognition" techniques for identifying any employee who may be misusing alcohol or drugs. Effective interviewing and counselling skills will be developed so that employees can be dealt with promptly, tactfully and firmly.
- All staff will be made aware of the effects of alcohol and drug misuse. They will be encouraged not to cover up for employees with a drink or drug problem but to recognise that collusion represents a false sense of loyalty and will in the longer-term damage those employees. Employees who recognise that they have a drink or drug problem, or that they are at risk of developing one, should be encouraged to come forward for confidential help. They should seek an appointment, in confidence, with the organisation's Medical Advisor.
- Contacts at outside agencies where help can be obtained will be made readily available to all members of staff via the organisation notice boards.

Reviewed by Marcus Kerridge McColl

December 2025

Annual Holiday Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To provide an extended period of leisure time, during which employees can have a break from work, relax and therefore, return to their jobs refreshed.

Scope

- All employees.

Policy

- It is the organisation's policy that employees should be given paid holiday every year, and that this holiday is taken.
- Employees should have the opportunity to take the holiday to which they are entitled.
- Managers are responsible for ensuring that the workload does not prevent this while also seeing that the needs of the organisation are taken into account.
- This policy does not form part of any employee's contract of employment and the organisation may vary or amend it at any time.

Procedure

OBTAINING APPROVAL

- All annual holiday must receive management approval before being taken. When you wish to take part of your annual holiday entitlement you should follow the procedure set out below:
 - All requests for annual holiday should be made in writing on the appropriate form (Holiday Request Form), at the earliest reasonable opportunity and generally no later than 4 weeks before the required start date. They should be passed to the Manager.
 - The Manager will consider the application, having regard for the needs of the organisation and the rules on annual holiday set out below. Where necessary, the Person Registered will be consulted before a final decision is made.
 - A copy of the form marked "approved" or "not approved", with brief reasons for refusal where applicable, should be signed by the Manager and returned to you. Only on receipt of this form, duly approved, should you commit yourself to any holiday plans. If you pay a deposit for the holiday, before the dates are approved, then the organisation is not responsible for any losses you may incur if the dates are refused.
 - In the event you dispute the decision of management, the organisation's Grievances Policy and Procedure should be used. Once approval for annual holiday has been given it will not normally be withdrawn.

CARRYING HOLIDAY OVER

- The organisation's holiday year runs for the period:
1st April – 31st March
- Your full annual holiday entitlement for the year must be taken, during the holiday year in which it accrues. No payment in lieu will be made for any untaken holiday, nor will you be allowed to carry it over to the next year in normal circumstances.
- Untaken holiday entitlement can only be carried over into another holiday year:
 - In cases involving long term sickness absence (see below);
 - In cases of maternity, paternity, adoption, parental or shared parental leave;
 - In exceptional circumstances, where your Manager has given permission in writing, or;
 - If otherwise required by law.

ENTITLEMENT

- Unless otherwise stated in an individual contract, you are entitled to

5.6 weeks

annual holiday per year (or the pro-rata equivalent if you work on a part time basis).
- 1 “week” is defined as the average working week of the twelve weeks prior to the holiday.
- If your employment starts or finishes part way through the holiday year, your holiday entitlement during that year shall be calculated on a pro-rata basis.

ACCRUAL

- In your first year of employment, and for leave purposes only, you accrue leave entitlement at one twelfth of your annual entitlement for each calendar month in which you work. Accrued entitlement for leave purposes is rounded to the nearest half-day. No service with any other employer counts for these purposes.
- Holidays may not usually be taken in advance of being accrued. If you wish to exceed this limit you must make your request to management at least 3 months in advance. If the holiday is approved, you are reminded that if you leave employment with the organisation having taken holidays in excess of your accrued pay entitlement, the excess will be reclaimed and deducted from your wages. In the event that the excess owed by you exceeds the wages owed, you must refund the difference to the organisation personally.
- For purposes of holiday pay due (or to be repaid) on termination, accrued holiday is calculated pro-rata to the actual days on which you were employed in the holiday year; irrespective of whether this is your first year of employment or not. For pay purposes there is no half-day rounding.

NEW EMPLOYEES

- Every effort will be made to meet the needs of new employees in respect of commitments to holidays already made. If you are a new employee you should inform the organisation of any holiday plans at the earliest opportunity. Where the booked holiday exceeds your annual holiday entitlement, consideration will be given to granting unpaid holiday up to a maximum of 2 weeks.

HOLIDAY RULES

- The following rules apply to all holidays:
 - Staff are unable to book holidays for two weeks over the Christmas and New year period.
- Where there are conflicting holiday requests, priority will normally be given to the employee whose request was received first. Management may exercise some discretion to ensure equitable treatment.
- No more than 2 weeks of annual holiday may normally be taken at one time. If you wish to exceed this limit you must make your request to management at least 3 months in advance.
- No more than 2 weeks holiday may be taken during the two quarters from April to September, and one each on the following two quarters. If you wish to exceed this limit you must make your request to management at least 3 months in advance.
- If you take annual holiday not previously approved you will be deemed to be absent without authorisation and you may be subject to disciplinary action, which could include dismissal.
- The organisation may require you to take (or not to take) holiday on particular dates, including when the business is closed, particularly busy, or during your notice period.
- If you are ill or injured whilst on holiday, to the extent that you would be unable to come to work (when you are meant to be at work) then you may choose to treat the period of incapacity as sick leave and reclaim the affected days of holiday. In these circumstances, you must comply with the organisation's sickness absence reporting requirements as if you were at work. If you are already on sick leave before a pre-arranged period of holiday, you may choose to cancel any days of holiday that coincide with the period of incapacity and treat them as sick leave. You will be entitled to any SSP (not holiday pay) in

relation to any such days of sickness. Any dishonest claims or other abuse of these rules will be treated as misconduct under the organisation's Discipline Policy and Procedure.

- Employees who are on sickness absence, including long term absence, continue to accrue statutory holiday entitlement. There may be limits to this as determined by law.

LATE RETURN

- If, for any reason, you know that you will be late returning from holiday, you must contact the Manager and notify them of your late return as soon as possible. The Manager will then discuss your options with you. Failure to do so will render you liable to disciplinary action for unauthorised absence.

LONG TERM SICKNESS ABSENCE AND HOLIDAY ENTITLEMENT

- Holiday entitlement continues to accrue during periods of sickness absence.
- If you are on a period of sickness absence which spans two holiday years, or if you return to work after sickness absence so close to the end of the holiday year that you cannot reasonably take your remaining holiday, you may carry over any unused holiday entitlement into the following holiday year. Carry over under this rule is limited to the four-week minimum holiday entitlement under EU law (which includes bank holidays), less any leave taken in the holiday year that has just ended. If you have taken four weeks' holiday by the end of the holiday year, you will not be entitled to carry on any leave over into the next holiday year.
- Any holiday that is carried over under this rule but is not taken within 18 months of the end of the holiday year in which it accrued will be lost.
- Alternatively, you can choose to take your paid holiday during your sick leave, in which case you will be paid at your normal rate of pay.

TERMINATION

- On termination of employment you may be required to use any remaining holiday entitlement during your notice period. Alternatively, you will be paid in lieu of any accrued but untaken holiday entitlement for the current holiday year to date. You are entitled to be paid at a rate of 1/260th of your full time equivalent salary for each day of untaken entitlement.
- If, upon termination of your employment, you have taken more annual holiday than you have accrued, the organisation reserves the right to deduct the equivalent of the payments made for such additional days from your final pay (calculated at 1/260th of your full-time equivalent salary for each excess day).

GUIDANCE TO THE SERVICE PROVIDER

- Although this policy refers only to employees, it applies equally to workers, who are also entitled to paid holiday under the Working Time Regulations 1998.

ROLLED UP HOLIDAY PAY

- "Rolled-up" holiday pay agreements with employees and workers (where holiday pay is paid as an identified mark-up on normal pay, rather than being paid when holidays are actually taken) have been challenged in the European courts and the practice is now unlawful.
- Although technically, this poses a risk of employees and workers alleging that they have not been paid for the holidays that they have taken (and therefore seeking to claim their holiday pay twice), some employers choose to take the risk of paying rolled-up holiday pay on the basis that it is simple and convenient.
- Case law suggest that any compensation awarded to individuals who claim is likely to be low or nil, provided that the rolled-up holiday pay arrangement is transparent, reflected in their contracts, and the holiday pay is genuinely in addition to normal hourly pay. If the rolled-up holiday pay is not in addition to the hourly rate of pay (i.e. an identified mark up) employers are at risk of claims under the Working Time Regulations and/or National Minimum Wage legislation.
- If a rolled-up holiday pay system is used, full holiday records should be kept and individuals should be encouraged to take all their holiday entitlement each year.

- The use of rolled-up holiday pay is a decision for the service provider to take, having regard to the fact that it has been held to be unlawful. Quality Compliance Systems Limited accepts not responsibility for claims or complaints arising from a decision to implement or continue a rolled-up holiday pay system.

CALCULATING HOLIDAY PAY FOR CASUAL WORKERS OR WORKERS WITH IRREGULAR HOURS

- The decision to use rolled-up holiday pay is often driven by the need to provide paid holiday entitlement to those whose hours, and sometimes periods of employment, are very irregular.
- The holiday entitlement for these staff, can be calculated as 12.07% of the hours that the work (then added as a mark up on their pay as part of a rolled-up holiday pay arrangement, or added to their accrued entitlement under a traditional, more lawful, holiday pay system).
- The 12.07% is calculated as follows:
 - 5.6 weeks (statutory minimum holiday entitlement) divided by 46.4 weeks (52 week full year minus 5.6 weeks) x 100 = 12.07%.
 - The 5.6 weeks is excluded from the year as the employee would not be present during the 5.6 spent on holiday in order to accrue annual leave.

CARRYING OVER ANNUAL LEAVE

- Where an employee or worker is on maternity, paternity or parental or adoption leave, he/she is entitled to accrue statutory and contractual leave in the normal way. The holiday pay accrued during that absence may be rolled over to the following holiday year.
- A woman on maternity leave must be able to take her paid annual leave during a period other than her maternity leave. This applies to the entire 5.6 weeks of annual leave under UK law. Employers therefore have a duty to allow the worker to take all of her annual leave before she goes on maternity leave, or allow her to carry it over into the next holiday year. ECJ case law suggests that where a period or parental spans two holiday years, the same applies.
- In case of sickness, employees and workers who are ill while on holiday to the extent that they would be unable to come to work (when they are meant to be at work) must comply with sickness absence reporting requirements as if they were at work. If they do this, they will be paid SSP and entitled to re-take their holiday at a later date (and carry it over into the next holiday year if they are unable or unwilling to take it in the current holiday year due to sickness). Carry over under this rule is limited to the four-week minimum holiday entitlement under EU law (which includes bank holidays) and not the 5.6 weeks of annual leave under UK law, less any leave taken in the holiday year that has just ended. If the employee has taken four weeks' holiday by the end of the holiday year, he/she will not be entitled to carry any leave over into the next holiday year.

DEDUCTIONS FROM SALARY

- Where annual holiday taken exceeds what has been accrued at the date of termination, you have the right to deduct the equivalent of the payments made for such additional days from employees' final pay ONLY if it is provided for, in writing and signed, in the employment contract.

Reviewed by Marcus Kerridge McColl
December 2025

Purpose

- To ensure that all employees comply with the Bribery Act 2010 and all its provisions, with special emphasis on (a) not giving bribes; (b) not receiving bribes.
- To clarify our responsibilities as an organisation in preventing bribery within the organisation.
- In particular it is our responsibility, when new employees are recruited, to bring the anti-bribery policy and procedure to their attention and ensure that they sign the anti-bribery declaration. It is our responsibility to explain and ensure that employees understand fully the measures of the law and the organisation's intention to ensure that it is being fully implemented.

Scope

- All directors, employees, contractors, agency staff, volunteers, interns, any agent or other person acting on our specific instructions. If you fall into any one of these descriptions then reference to "you" in this policy refers to you.

Policy Background

- In guidance issued by the Ministry of Justice, a bribe is defined as: "Giving someone a financial or other advantage to encourage that person to perform their functions or activities improperly or to reward that person for having already done so". It goes on to state that "this could cover seeking to influence a decision-maker by giving some kind of extra benefit to that decision maker rather than by what can legitimately be offered as part of a tender process".
- So it includes:
 - Gifts;
 - Hospitality;
 - Facilitation payments;
 - Contributions to political organisations and charities.
- Crescent Community Care Services Limited can be liable under the Bribery Act if you commit a bribery offence. This is because your activities can be attributed to the organisation.
- You can also be punished as an individual by up to ten years imprisonment and a fine. If we as an organisation have taken part in corruption we could face an unlimited fine and considerable damage to our reputation. We therefore take our responsibilities very seriously.

Your Responsibilities

- It is your responsibility to read and be confident that you understand and fully comply with this policy. In any area of doubt refer to the Registered Manager.
- You are required to notify the Registered Manager if you suspect a breach of this policy has occurred.
- You will not suffer detriment as a result of any matter raised, in good faith, under this policy.
- However if you personally breach this policy then we may take disciplinary action (including possibly dismissal for Gross Misconduct) against you.
- If you are not an employee, we may terminate your contract with or without notice as appropriate.

What constitutes a bribe?

- If someone seeks to influence you to act so that you breach trust, lack impartiality or perform in bad faith (i.e. act improperly) then you need to be on your guard. If they seek to do so by offering, promising or giving an advantage to you (such as money, a gift, or a legacy) then that is bribery. Bribery also occurs if you request, agree to receive or actually receive such an advantage. Such exchanges are illegal.
- If you pay, or attempt to pay cash or any other favour or consideration in order to secure business (e.g. a new source of service user), retain business or gain a business advantage for us then that is a bribe. It is a specific offence to bribe or attempt to bribe a public official (such as a CQC Inspector) wherever they may be in the world.
- Reasonable and proportionate gifts as a token of appreciation or as part of facilitating normal business relationships are acceptable; the key is that they must not be an inducement to act improperly.
- If you influence or make decisions on our behalf you are more vulnerable and if you are uncomfortable with any form of influence you should report it to the Registered Manager.

- Offering or accepting gift or hospitality needs caution; in particular such offers and acceptance need to be for items of proportionate value.
- Typical examples of gifts of proportionate value as appreciation would be a bottle of wine, flowers, or chocolates either at Christmas or as a genuine “thank you”. These are normally acceptable. However if they are a form of facilitation payment (see below), then they are not.
- All gifts should be declared to the Registered Manager and a record will be kept.
- Hospitality is defined as where we are present at the hospitality itself (say a buffet lunch at an exhibition). Where this level of hospitality falls within our normal business relationship this is acceptable. Hospitality which is disproportionate may be seen to be influencing impartiality and is prohibited under this policy. Please refer any such offers or concerns to the Registered Manager.
- Where the company providing the hospitality is not present, for example if a restaurant voucher is given, then this is a gift, not hospitality. If you are in doubt about the proportionality of any gift you should discuss the matter with the Registered Manager.
- If you are involved in any form of tendering process then gifts and hospitality (even if proportionate) must not be accepted and, if offered, should be politely declined and referred to the Registered Manager.
- Facilitation payments may be experienced where a relative or other agent offers a payment to obtain a level of service for a user that would not normally be provided. Our absolute policy is that such payments are not to be accepted and should be reported to the Registered Manager.
- Requesting a facilitation payment may be regarded as Gross Misconduct and render you liable to summary dismissal.
- If there is the slightest suggestion that you might receive a legacy from a service user then this should be referred to the Registered Manager immediately. Legal advice will be sought.
- You may not solicit gifts or sponsorship for charitable events from residents.
- No donation to a Charity or Political Party/Candidate may made or offered in the name of the organisation without the prior approval of the Registered Manager.
- You should avoid any situation where you might benefit personally from decisions you take or influence on behalf of us. Where a conflict of interest could arise the relevant decision must be referred to the Registered Manager.

Raising Concerns

- You should raise any concerns that you have about wrong-doing under the Whistle Blowing policy, following the procedure there.
- Ultimate responsibility for compliance rests with the Company Secretary.

Guidance to the Service Provider

- An organisation can also be found liable where someone who performs services for it, such as an employee or an agent, pays a bribe in order to secure business, retain business or gain a business advantage for the organisation in question. A defence is in place should this happen, but the organisation needs to be able to prove that it has adequate procedures in place in order to prevent bribery.
- What counts as “adequate” within the defence will depend on the bribery risks you face, e.g. the nature of your business, the size of your organisation, complexity of your business etc. If your business is therefore small, then the procedures you will need to have in place will need to be minimal in order to mitigate those risks, but if you are a large organisation then the risks will be exponentially greater and you should therefore have detailed, extensive procedures in place.
- Larger organisations may be wise to assess their risks carefully and to obtain from employees, particularly those who may be in a position to accept or offer bribes, a signed declaration that they have read, understood and accept the above procedure.
- In any event, you should bring the above policy to the attention of employees. For very small organisations it may be sufficient to do this orally.

Principles

The following six principles will help you to decide what needs to be done for your particular organisation:

- **Proportionality**
 - The action which you decide to take must be proportionate to the risk that you might face and to the size of your business. Consequently you may need to do more if your business is large. Most independent care sector organisations will be operating in a market place where the risk of bribery is small, therefore your actions may not need to be extensive.
- **Top Level Commitment**

- Senior management are in the best position to ensure that the organisation conducts its business without bribery, and should enforce this with a top-down approach. The senior management (especially the Registered Manager and the Registered Provider) will therefore want to actively demonstrate that they have made all staff aware of their intolerance of bribery.
- **Risk Assessment**
 - Think about the people you deal with, especially new business arrangements and public funding bodies, and the inherent risks involved.
- **Due Diligence**
 - Knowing exactly who you are dealing with and employing can help to protect the organisation from less trustworthy individuals. It is therefore especially important that the results of DBS checks are thoroughly checked as part of the recruitment procedure, so that risks are taken into account when making employment offers.
- **Communication**
 - Communication to care staff and representatives of your policies and procedures will assist in enhancing the awareness of how you like to conduct business. In most cases it will be sufficient to draw members of staff's attention to your policy as already discussed.
- **Monitoring and Review**
 - The risks the organisation faces and the effectiveness of its procedures may change over time. You may therefore want to review the latest anti-bribery steps you have taken, so that they keep pace with any changes in the risks that you may face.

Compliance

- It is assumed, within this policy, that employees will seek guidance from the Registered Manager in areas of uncertainty. This is not to suggest that that person will necessarily have the correct answer but responsibility should be placed on him or her or on another specified responsible person to take responsibility for finding the answer or providing appropriate guidance. There is a significant amount of guidance, about proportionality for example, in Government publications on the Internet and in other reliable web sources.
- It is important that one individual takes responsibility for day-to-day compliance. In most cases this would naturally fall to the Registered Manager but ultimate responsibility will fall on the Company Secretary, Owner or an equivalent position.

Hospitality

- Genuine hospitality or other similar business expenditure that is reasonable and proportionate, is not prohibited under the Act. All hospitality must however, be *bona fide*.
- Should hospitality be considered a cover for bribery, then the authorities will look at the value of the hospitality, the level provided, the way in which it was provided and the influence of the person receiving it on a business decision.
- The guidance from the Ministry of Justice does state that a business can continue to provide genuine hospitality, such as taking clients to sporting events, dinners, offering gifts to clients as a reflection of your good relations or paying reasonable travel expenses. For example, it may be acceptable to accompany a potential client to a sports event and to then provide dinner. However to provide them with tickets to go on their own, or with others unconnected to your business, and to then also pay for their dinner and travel expenses could be seen as a gift and, if disproportionate, as a bribe.
- No gifts or hospitality should be given or accepted during a tender process or during contractual negotiations if there is any realistic risk that such gifts or entertainment could influence the outcome of the tender process or contractual negotiations.

Facilitation Payments

- Facilitation payments, which are payments to induce officials to perform, or speed up the performance of routine functions that they are otherwise obligated to perform, are bribes. An example here would be attempting to bribe a CQC inspector. There is no exemption for such payments under the law.

Political Contributions

- It is unwise to make donations, whether in cash or kind, in support of a political party or candidate, as this can be perceived as an attempt to gain an improper business advantage.

Charitable Contributions

- Charitable support and donations are acceptable under law whether of in-kind services, knowledge, time, or direct financial contributions. However, service providers must be careful to ensure that charitable contributions are not used as a cover to conceal bribery. Only make charitable donations that are legal and ethical.

Reviewed Marcus Kerridge-McColl
September 2025

Anti-Bullying Policy and Procedure Crescent Community Care Services Limited

Scope

- All workers and Service Users.

Policy

- We are committed to providing a caring, friendly and safe environment for all of our workers and Service Users so they can work and live in a relaxed and secure atmosphere. Bullying of any kind is unacceptable, and if bullying does occur, all incidents will be dealt with promptly and effectively.
- The attention of all employees is drawn to the:
 - The Social Care Workers Codes of Practice, which can be found for download on the Social Care Institute of Excellence (SCIE) website at <http://www.scie.org.uk/workforce/files/CodesofPracticeforSocialCareWorkers.pdf?res=truea> copy of which is given to all employees;
 - Discipline Policy and Procedure, a copy of which is shown to all employees on recruitment, is referred to in the Staff Handbook that is issued to all employees, and is available in the Policy and Procedure Manual in the staff room/administration office. This policy sets out the disciplinary action which may be taken in the event of bullying activity;
 - Whistleblowing Policy and Procedure, a copy of which is shown to all employees on recruitment, is referred to in the Staff Handbook that is issued to all employees, and is available in the Policy and Procedure Manual in the staff room/administration office;
 - Sexual and Racial Harassment Policy and Procedure, a copy of which is shown to all employees on recruitment, is referred to in the Staff Handbook that is issued to all employees, and is available in the Policy and Procedure Manual in the staff room/administration office.
- What is bullying?
 - Bullying is the use of aggression with the intention of hurting another person physically or emotionally.
 - Bullying results in pain and distress to the victim.
 - Bullying can be:
 - Emotional; being unfriendly, excluding and ignoring, tormenting (e.g. hiding or interfering with belongings, threatening gestures, threatening language);
 - Physical; pushing, kicking, hitting, punching or any use of violence;
 - Racist; racial taunts, graffiti, gestures;
 - Sexual; unwanted physical contact or sexually explicit comments;
 - Homophobic; because of, or focusing on the issue of sexuality;
 - Verbal; name-calling, criticising in public, sarcasm, spreading rumours, teasing;
 - Cyber; All areas of the internet, such as email and internet chat room misuse;
 - Mobile; threats by text messaging and calls, misuse of associated technology i.e. camera and video facilities.
- Signs and symptoms.
 - A person may indicate, by signs or behaviour, that he or she is being bullied. Supervisors should be aware of these possible signs and that they should investigate if a person:
 - Is reluctant to work without a specific person accompanying them; ☹
 - Asks that a particular member of staff does not care for them;
 - Changes their usual routine;
 - Is unwilling to come to work;
 - Begins to take unannounced days off;

- Becomes withdrawn, anxious, or lacking in confidence;
 - Feels ill in the morning;
 - Begins to work to a lower standard;
 - Has possessions which are damaged or 'go missing';
 - Has unexplained cuts or bruises;
 - Becomes aggressive, disruptive or unreasonable;
 - Is bullying other people;
 - Is reluctant to discuss their behaviour;
 - Gives improbable excuses for any of the above;
 - Is nervous and jumpy when a cyber message is received.
- These signs and behaviours could indicate other problems, but bullying should be considered a possibility and should be investigated.

Procedure

- Report bullying incidents to your supervisor.
- Record cases of serious bullying.
- If necessary and appropriate, police will be consulted.
- The bullying behaviour or threats of bullying must be investigated and the bullying stopped quickly.
- An attempt will be made to help the bully/bullies change their behaviour.
- Use the supervision process to formally record bullying behaviour, and agree a plan of action to stop it.
- Clearly spell out the potential disciplinary action which could be taken in the event of appropriate corrective action not being followed by a person who is exhibiting bullying behaviour, or if serious incidents occur.
- Refer to the Discipline Policy and Procedure in serious and/or intransigent cases.
- Use the abuse procedures in the case of bullying of a Service User.
- Consider the use of advocacy in the case of a Service User bullying a worker, and refer the Service User and/or their advocate to the behavioural standards set out in the Service User's Handbook and Agreement.

Reviewed by Marcus Kerridge McColl
December 2025

Anxiety Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To provide individualised, safe and supportive care for people with a diagnosis of anxiety or people who may live with anxiety.

Scope

- All workers working with Service Users who have been given a diagnosis of anxiety disorder or who may be living with an anxiety disorder.

Definition

- Anxiety is a term in very common usage. It is usually described as a feeling of fear that is not rational. The symptoms may be physical, such as sweating, becoming dry mouthed, butterflies in the stomach, and others. There may be symptoms in the persons thinking, poor concentration, or feeling that something awful will happen.
- These symptoms may, for some, just last a short time, but for others much longer. The psychiatric classification system, Diagnostic and Statistical Manual of Mental Disorder volume four (DSM-IV) is widely used in mental health services in the United Kingdom, and defines anxiety by identifying seven types:
 1. **Panic disorder with or without agoraphobia** - The main characteristic of panic disorder is the occurrence of a panic attack linked with the fear of another attack.
 2. **Phobias** - Specific phobias and social phobia are both included under this heading. A phobia is described as an intense and irrational fear of a specific object or situation that is so intense it can cause the individual to be compelled to go to great lengths to avoid it. Social phobia can include a fear of being judged, scrutinised or humiliated in some way.
 3. **Obsessive Compulsive Disorder** is characterised by unwanted, intrusive, persistent or repetitive behaviours. These will reflect a person's attempts to control them and the anxiety caused by them.
 4. **Stress disorders** - This includes post-traumatic stress disorder (PTSD) and acute stress disorders. These are categorised as being a symptomatic response to a traumatic experience that the individual has had.
 5. **Generalised Anxiety Disorder (GAD)** is the most commonly diagnosed anxiety disorder and usually affects young adults.
 6. **Anxiety disorders due to known physical cause** - This includes medical conditions and symptoms caused by drug misuse.
 7. **Anxiety disorder not otherwise specified** - This is not a separate type of disorder but included in order to cover symptoms that do not meet the criteria for any of the other anxiety disorders.
- Diagnosis of anxiety is undertaken by medical practitioners. It is regarded as a common health problem, in that the symptoms of anxiety disorders may be a common experience amongst the general population. Care and support staff may be working with someone who they feel has symptoms of an anxiety disorder. Consideration should be given to referring that person to their General Practitioner. Signs and symptoms that care and support staff should be alert to include the following:

Social

- Taking extreme actions to avoid situations (like never going outside).
- Being fearful of meeting people.

Physical Impact

- Not sleeping properly.
- Physical feelings of anxiety such as dry mouth or stomach cramps.

Psychological

- Panic attacks, unpredictable and sudden attacks of anxiety.
- Poor concentration.
- Irritability.
- Feeling worried a lot of the time.

Description

- As with many psychiatric diagnoses, the term anxiety can be a controversial one. Anxiety can be a perfectly usual response to a number of situations, particularly in a modern busy stress-laden society. However, where the symptoms of anxiety become so disabling for someone that it affects their daily living then this suggests the person is in need of support to manage this.

Policy

- There are a number of good practice principles that care and support staff should be aware of whilst providing care and support for people with anxiety disorder:
 - Being aware that a mental health diagnosis can be stigmatising, and that people may not want to admit to this;
 - Being able to identify other sources of support, including self-help groups, support groups and other local and national resources;
 - Awareness of the principles of the Mental Capacity Act 2005 (MCA). QCS policies on the MCA will provide further information when working with people who lack mental capacity to make decisions about their care;
 - Being aware that there are different types of anxiety, and recovery is possible.

Procedure

- The National Institute for Health and Clinical Excellence (NICE) has produced guidelines for treating and managing certain types of anxiety. It covers psychological treatments, medication and the services that should be made available in hospital and in the community. These guidelines are reviewed regularly and are downloadable from the NICE website at www.nice.org.uk. Specifically, there are NICE guidelines published on the following types of anxiety disorder:
 - Generalised Anxiety Disorder (CG113);
 - Social Anxiety (CG159);
 - Obsessive Compulsive Disorder (CG31);
 - Post-Traumatic Stress Disorder (CG26).
- For the majority of people who are diagnosed with an anxiety disorder, interventions identified in the NICE guidelines will be delivered by their GP and primary care team. For people with the most severe and disabling disorders, those interventions are likely to be carried out by specialist mental health services. However there is still a crucial role for staff working in care home and community settings in supporting these interventions. Some of these ways are described in NICE guidelines, and the ways that care staff can support people with anxiety disorders could include the following:
 - Build a relationship and work in an open, engaging and non-judgemental manner;
 - Explore the person's worries in order to jointly understand the impact of their anxiety;
 - Explore treatment options collaboratively with the person;
 - Ensure that discussion takes place in settings in which confidentiality, privacy and dignity are respected;

- Provide information appropriate to the person's level of understanding about the nature of their disorder, or sources of further information including self-help groups and support groups, in particular where they can talk to others with similar experiences;
- When families and carers are involved in supporting a person with anxiety, provide information on opportunities to seek a carer's assessment of their caring, physical and mental health needs and information about carer support groups.
- Specialist interventions that might be offered to someone with anxiety disorder will include:

Self-help

- Self-help groups have provided a lot of support for people with anxiety, and may offer support using principles of Cognitive Behavioural Therapy. Organisations such as Anxiety UK can offer information on local groups. GP surgeries are another source for further information on local support groups.

Talking treatments

- These might include counselling and Cognitive Behavioural Therapy (CBT) to understand why you think the way you do. These treatments would be arranged via someone's General Practitioner, and in England may be provided by the NHS Improving Access to Psychological Therapies programme.

Medication

- There are different types of anti-anxiety medication. These can include tranquilisers and anti-depressant medication. Some of these may be addictive, and some may have side effects and may take some time before they help with symptoms. The person's GP would advise on the prescription of these medications.

Crisis intervention

- Where there is need for urgent intervention, particularly where someone is expressing suicidal intentions, then in such situations the involvement of Crisis Resolution and Home Treatment (CRHT) teams may be required and access to crisis intervention can be made through referral by the person's GP.

Frameworks for delivering care and support

- All treatments and interventions should be offered as part of a care planning process based on an individual's needs. Tools in the QCS Care Management System allow for compilation of assessment of mental health needs and a care plan format that should be reviewed on a regular basis. The majority of people receiving treatment for anxiety will receive this from their General Practitioner. For those whose mental health needs are complex and severe, who receive care and treatment from mental health services, this should be delivered as part of the Care Programme Approach framework for delivering mental health care. The support and interventions by care staff should be included in the overall Care Programme Approach care plan, and these staff may be invited to contribute to CPA care planning meetings. The person who is the subject of the CPA care plan should have their own copy. Where care staff are aware that someone is in receipt of professional mental health services, they should be aware of how the care plan can be accessed, and who the person's care co-ordinator is. If not they should contact the appropriate mental health team. The contents of a CPA care plan should include:
 - Details of who the care co-ordinator is (mental health professional);
 - Identify the interventions and anticipated outcomes;
 - The date of the next planned review;
 - Crisis and contingency arrangements;
 - Arrangements for care and support;
 - Arrangements for the management of risk to the Service User and to others.

- Crisis and contingency planning would be conducted by the person's care co-ordinator, but care staff should be aware of these plans. A crisis plan should identify responses to crisis situations that might arise for that person, such as a worsening of their anxiety disorder that cannot be managed with usual coping strategies. A contingency plan will outline a planned course of action that should follow some breakdown or failure in the person's care plan. Contact names and numbers of key professionals should form part of these plans, but a list of names and numbers on their own would not be regarded as sufficient. For crisis and contingency plans to be effective and workable, the Service User should be involved in drawing up their own plan when they are experiencing a period of wellness.

Risk Management

- If you believe someone you are working with is at risk of harm you should share that information with others you work with, and where appropriate, the person's GP. Assessment and management of people at risk is a specialist area of work that should be undertaken by health professionals, but care staff and the person themselves should be able to contribute to that. Risk management should not be seen as elimination of risk. There are risks for everyone in all walks of life, and to try and eliminate them would result in a loss of independence and choice. So risk management should be seen in the context of positive risk taking. Current policy thinking in relating to people with mental health problems can be found in the Department of Health's Best Practice in Managing Risk (2007). The Best Practice document also describes the importance of a collaborative approach to managing risk, involving the person and the whole of the care team, so that trusting relationships are developed that aid communication.

Legal Frameworks

- The framework for the compulsory care and treatment of people with mental disorder is governed by the Mental Health Act 1983 (which was amended in 2007). The Mental Health Act Code of Practice gives guidance on the definition of mental disorder for the purposes of the Act. The Code includes the following as examples of conditions which could fall under the definition of a mental disorder:
 - *Neurotic, stress-related and somatoform disorders, such as anxiety, phobic disorders, obsessive compulsive disorders, post-traumatic stress disorder and hypochondriacal disorders.*
- Where a person lacks capacity as a result of an impairment of, or a disturbance in the functioning of, the mind or brain, to make decisions, the principles involved in making decisions for someone in their best interests are governed by the Mental Capacity Act 2005. The Mental Capacity Act 2005 also includes provision for the appointment of lasting powers of attorney, regarding financial and welfare decisions if the person should lose the mental capacity to make these decisions.
- Policy and procedures surrounding the Mental Health Act 1983 and the Mental Capacity Act 2005 are contained within the QCS manuals.

Reviewed by Marcus Kerridge McColl
November 2025

Appearance Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To maintain a professional impression

Scope

- All staff representing our company

Policy

- All staff members are required to be clean and tidy in appearance.

- Staff members own personal care must be to a high level as you are working closely with both Service Users and other Staff members.

Procedure

- During the Policies and Procedures training the Manager will outline the standards of personal care and standards of dress which apply to staff as follows
 - Care Workers are to wear the appropriate Uniform at all times
 - Should have good personal care themselves
 - Clothing must be clean, odour free and in good condition
 - Hair must be kept clean and tidy
 - Staff must keep their hair tied back so not to cause a Health & Safety issue.
- Jewellery / Badges etc – Crescent Community Care Services Limited reserves the right to require that employees do not wear jewellery including wrist watches, badges or clothing that contains text or images which it believes may cause offence to the Service User or other members of staff
- Should any employee have difficulties in maintaining these standards they should discuss the matter with their immediate superior
- Employees who do not comply with these standards will be subject to the organisations normal disciplinary procedure. In serious cases, where an employees appearance is, in the organisations view, unacceptable, the employee will be required to return home to change.
- Employees have the right to lodge an appeal against a decision on the acceptability of their appearance, using the organisations normal grievance procedure.
- The organisation reserves the right to change the uniform as necessary in order that it conforms with the Corporate Image.
- Failure to wear the organisations uniform, or to keep it in the proper condition, will be treated as an issue of misconduct and dealt with under the normal Disciplinary Procedure.
- PPE (Personal Protective Clothing & Equipment)is essential and to be used with the uniform
 - For care staff this includes
 - Aprons Where appropriate
 - Gloves at all times
 - Flat & closed in shoes at all times
- The PPE here is specified as a minimum. Where risks can be reduced by reasonable and additional protection, further PPE may be required.
- Where PPE is provided, this must be worn at all times. Any failure to do so places an individuals safety at risk and may result in disciplinary action. All members of staff that are issued PPE should keep it maintained and only use it for the companies purposes.
- All uniforms must be returned on leaving and where uniforms are not returned in reasonable condition a charge will be made to cover the cost of replacement.

Reviewed as Marcus Kerridge-McColl
 December 2025
 Blood Spillage Policy & Procedure
 Crescent Community Care Services Limited

Purpose

- To prevent cross contamination from Exposure to blood

Scope

- All Spillages of blood

Policy

- Blood spillage from any source will be cleaned in a manner which protects the person cleaning the spillage from the possibility of cross contamination

Procedure

- Materials for dealing with all blood spillage

- 1 disposable plastic apron
- 1 pair of disposable gloves
- 1 yellow bag
- Paper towels
- Bowl
- A sodium Hydrochlorite 1% solution (i.e Milton Fluid or the same)
- All blood spillages / splashes must be dealt with immediately
- Careful dilutions of bleach water or Milton Fluid
- Pour the cleaning solution into the bowl for cleaning
- Wear disposable gloves and apron, wipe over the area with paper towels until all of the blood has been removed and then place towels into the yellow disposal bag
- Wash the are with paper towels and the cleaning solution and then place into the yellow disposal bags
- Wipe over the area to ensure that the area is clean
- Wash away the solution and ensure the sink is cleaned out fully with cleaning fluid
- Secure the yellow disposal bag and inform for collection correctly

NOTE

For the most, Care workers should not clean up blood spillages unless in a real emergency where the above procedure is carried out.

Reviewed Marcus Kerridge-McColl
October 2025

Care and Daily Living Policy and Procedure
Crescent Community Care Services Limited

Purpose

- The daily living experience of Service Users will be enhanced by the skilled and competent delivery of Care.

Scope

- All Care activities.

Policy

- Care training to best practice standards will be carried out.

Procedure

- Assessments of the Service User's health and daily living needs will be carried out by the person nominated by the Manager for that purpose.
- An evaluation and review of the Service User's personal health status will be carried out by the person nominated by the Manager for that purpose.
- An evaluation of the Service User's personal evaluation of their own wellbeing will be carried out by the person nominated by the Manager for that purpose.
- Service User satisfaction surveys which incorporate Service Users' opinions of their Care will be carried out.
- Audit and review of the Service User Care Plans and achievement of Service Users' outcomes detailed in the Daily Record documentation will be carried out, together with personal interviews with Service Users.
- Staff tasked with any of the above will be appropriately skilled and qualified.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To support the career development of adult social care workers who are not members of regulated professions.

Scope

- All adult social care workers who are not members of regulated professions.

Policy

- The Care Certificate replaces previous standards for induction training and development for adult social care workers.
- As an induction standard, the implication is that all workers should be benchmarked against the Care Certificate standards, and where skills and knowledge are deficient, topped up by training and development. In this manner, the whole workforce will be working to common standards of competence in the areas covered by the Care Certificate, with substantial benefits to the quality of the services offered to Service Users.
- Because of the potential benefits to staff competence and consistency of working, which in turn will significantly improve quality of services to Service Users, QCS recommends that:
 - All newly recruited adult social care workers, who are not members of regulated professions, complete the Care Certificate.
 - All existing workers' competencies and skills are assessed against the Care Certificate standards, and where any deficiencies are identified, those deficiencies are added to the personal development plan for that individual worker. Note that it may be organisationally more convenient for the Care Certificate development to be offered to all workers. Some workers may prefer to be seen to be up to date with their certified training for either personal or professional reasons.
- Crescent Community Care Services Ltd will support all employees to complete a Care Certificate, providing access to either internal or external resources in order to do so, as best suits the organisation and the individual.
- Note that the Skills for Care documentation uses the word "worker" throughout, which indicates that, in line with the QCS training approach, all employees are subject to the induction standards, and not only those who are direct carers. This approach recognises the team, approach to quality care, the need for the motivation of all workers including indirect workers, and the fact that indirect workers do have an impact on perceived care quality.

Background

- The Care Certificate is a set of knowledge and skill standards which is designed to apply to workers in adult social care who are not a member of regulated profession. This group includes:
 - Activities worker;
 - Day Care Assistant, Day Care Officer;
 - Domiciliary care worker, Home care worker;
 - Nursing Assistant (in a nursing home or a hospice);
 - Personal Assistants, Reablement Assistant;
 - Residential Care Worker, Senior Home Care Worker;
 - Support Worker.
 - Other roles may be included where achievement of all of the standards is possible.
- Quoted from Skills for Care: (<http://www.skillsforcare.org.uk/Standards/Care-Certificate/Care-Certificate.aspx>)

- Following the Francis Inquiry the Secretary of State has issued recommendations on the recruitment, learning and development, management and support of healthcare assistants and social care support workers.
- The Care Certificate is the start of the career journey for these staff groups and is only one element of the training and education that will make them ready to practice within their specific sector. Although the Care Certificate is designed for new staff, new to care and offers this group of staff their first step on their career ladder, it is also offers opportunities for existing staff to refresh or improve their knowledge.
- The Care Certificate sets out explicitly the learning outcomes, competences and standards of care that will be expected in both sectors, ensuring that the HCSW/ASCW is caring, compassionate and provides quality care.

THE STANDARDS

1. Understand Your Role

- The QCS recruitment process, job descriptions and the recommended availability of all policies and procedures to all employees supports this element of the standards.

2. Your Personal Development

- Use of the QCS Training Policy and Procedure approach, with the development of personal development plans supports this element of the standards.

3. Duty of Care

- The recommended availability of all QCS policies and procedures to all employees supports this element of the standards.

4. Equality and Diversity

- The recommended availability of all QCS policies and procedures to all employees supports this element of the standards.

5. Work in a Person Centred Way

- The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure, and the Service User Handbook, to all employees supports this element of the standards.

6. Communication

- The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure, and the Service User Handbook, to all employees supports this element of the standards.

7. Privacy and Dignity

- The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure and the Service User Handbook, to all employees supports this element of the standards.

8. Fluids and Nutrition

- The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure and the Nutrition Policy and Procedure, to all employees supports this element of the standards.

9. Awareness of Mental Health, Dementia and Learning Disability

- The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure, and the Service User Handbook, to all employees supports this element of the standards.

10. Safeguarding Adults

- The recommended availability of all QCS policies and procedures, particularly the Safeguarding Policy and Procedure, the Care Planning Policy and Procedure and the Service User Handbook, to all employees supports this element of the standards.

11. Safeguarding Children

- The QCS Safeguarding Policy and Procedure supports this element of the standards.

12. Basic Life Support

- To meet this element of the standards, it is necessary for employees to successfully complete practical Basic Life Support Training that meets the UK Resuscitation Council guidelines. The guidelines can be found at <https://www.resus.org.uk/pages/guide.htm>. While training in this area is an important element of the skills and knowledge of an adult social care worker, and may come to be expected by Service Users and their families, it is a specialised area and staff development should be through recognised and certified trainers, with formal certification of competence at the end of the training, and regular re-certification.

13. Health and Safety

- The recommended availability of all QCS policies and procedures, particularly the Health and Safety Policy and Procedure, to all employees supports this element of the standards.

14. Handling Information

- The recommended availability of all QCS policies and procedures, particularly the Communication Policy and Procedure, the Confidentiality Policy and Procedure and the Care Planning Policy and Procedure, to all employees supports this element of the standards.

15. Infection Prevention and Control

- The recommended availability of all QCS policies and procedures, particularly the Infection Control Policy and Procedure, to all employees supports this element of the standards.

Procedure

- New recruits
 - Make it clear that new recruits will be expected, as a condition of their employment, to complete the Care Certificate.
 - In cooperation with the new recruit, assess the method of development which best suits them and the organisation, for instance, self-study, or external courses.
 - Support the new recruit to complete the Care Certificate.
- Existing employees
 - Encourage all existing employees to work to obtain a Care Certificate.
 - In cooperation with the employee, assess the method of development which best suits them and the organisation, for instance self study, or external courses.
 - Support the employee to complete the Care Certificate.
- Enabling Care Certificates for all employees:
 - Decide as an organisation how you can best support development of the workforce towards Care Certificates. While the Care Certificate is ideally suited to the in-house development method, which is proven to be the most effective model for most employees, the support for that in-house development must be designed, planned and implemented for results to be tangible. Implementation methods may include the training of in-house trainers and assessors (usually regarded as the most effective method), or combining in-house assessors with external teaching aids such as eLearning assessors and external teaching aids.
 - Skills for Care provide a range of materials, including workbooks, which effectively support an in-house development model. These, and other resources, are available at: <http://www.skillsforcare.org.uk/Standards/Care-Certificate/Care-Certificate-Materials.aspx>

- The CQC's approach to Care Certificates is set out at:
http://www.cqc.org.uk/sites/default/files/20250318_one_page_cqc_position_care_certificate.pdf

Reviewed by Marcus Kerridge McColl
December 2025

Care Communication and Information Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To support the values of The Agency.

Scope

- All Care procedures.

Policy

- Continuity of Care delivery will be based on the unique, holistic needs of each Service User, enabled by the effective recording of information.
- Service Users will have access to their own Service User Care Plan.

Procedure

- All information related to the Service Users' health and social wellbeing is documented in the Service User Care Plan.
- Service User Care Plans are current and appropriate and easily understood.
- All relevant information is readily available and accessible.
- All staff will have read the Service User Care Plans before writing the record of daily Care.

Residential Setting

- For all staff working in a residential setting, any changes to Service Users are monitored and documented in the communication book and the Service User Care Plan.
- All staff must read the communication book before commencing duties, reading all entries made since the last time they were on duty.
- The attached Care Communication and Action Form will be used to communicate with Care administration, the Care supervisor or the Registered Manager.

Community Setting

- For all staff working in a community setting, any changes to Service Users are monitored and communicated to the administration office. This action should be noted by the staff member in the record of daily care, and in the Service Users Care Plan.
- All staff must read the record of daily Care before commencing duties, reading all entries made since the last time they were on duty.

- On receipt of the changes, the Administration staff must complete the attached Care Communication and Action Form.

Reviewed by Marcus Kerridge McColl
December 2025

Care Management Protocol
Crescent Community Care Services Limited

Purpose

- To define accountability, responsibility and lines of communication and authority.

Scope

- All workers.

Policy

- The day-to-day management of Care within The Agency, including compliance with the organisation's policy, legislation and best practice, is the responsibility of the Registered Manager. The Registered Manager is responsible to the Registered Provider for the standard of the Care in The Agency.
- The Registered Manager will carry out day-to-day quality control of care, ensuring that all organisational policies, statutory requirements and best practices are complied with, and that changes to promote continuous improvement are supported.
- The Registered Manager will carry out regular Care Audits and report the results to the Management Meeting, together with:
 - Inspection reports;
 - Accident reports;
 - Customer feedback and complaints.
- No changes to the Care system, service, equipment, staffing, suppliers or supplies specification may be made without consultation with, and the agreement of, the Registered Provider.
- The Registered Manager will have 2 years experience in a senior management capacity in a relevant Care setting within the past 5 years, and a qualification to at least QCF Diploma Level 4 or equivalent, or, in the case of a nursing home, be a Registered Nurse and have a relevant management qualification.
- The Registered Manager will only be responsible for one registered establishment.

Reviewed by Marcus Kerridge McColl
December 2025